



COLLEGE
OF
MEDICINE

Abdominal pain in pregnancy

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physiology

- Pain is produced in different tissues and it is protective mechanism for the body.**
- It occurs whenever any tissues being damaged, and it cause the individual to react to remove the stimulus.**
- Three types of stimuli excite pain receptors:**
 - Mechanical**
 - Thermal.**
 - Chemical.**



Abdominal Pain In Pregnancy

- Abdominal pain in pregnancy is often due to a benign cause ,while there are many harmless causes but there may be serious cause.
- Abdominal pain in pregnancy may be a normal part of the process as body changes as to accommodate growing baby .



Benign cause:

- Round ligament pain
- Gas and constipation
- Braxton Hicks contraction
- Common discomfort related to
- stomach, kidney, growing uterus ,etc...



Causes of abdominal pain in pregnancy

1. Pregnancy related

• -Early pregnancy:

- 1.ectopic pregnancy.*
- 2.miscarriage*

-Later pregnancy:

1.labour (term or preterm).

2.placental abruption.

3.uterine rupture.

4.Acute Polyhydramnios

5.Choioamniotitis, usually precipitated by pre-term premature rupture of membranes



2. Non pregnancy related ,gynecological causes:

- 1. Ovarian cysts or tumours, including rupture, haemorrhage or torsion***
- 2. Red degeneration of a fibroid***
- 3. Uterine torsion***



3. Surgical causes

a. Abdominal wall, especially symphysis pubis and ligament strains.

b. Gastrointestinal causes:

1. Constipation

2. Appendicitis:

-affects 1 in 1500 pregnancies, less common than in non-pregnant women.

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- Diagnosis is complicated by change in position of appendix which is carried high and to the right and there may not be localization of pain.**
- Nausea, vomiting, fever together with any right-sided abdominal pain are suspicious of this.**
- Leucocytosis is suggestive.**
- Appendicitis is not diagnosed in 1 in 5 cases in pregnant women until the appendix has ruptured causing peritonitis, which can cause premature labour or abortion**



3.Perforated peptic ulcer, gastritis

4.Bowel obstruction, e.g. adhesions, volvulus

5.Gallstones, cholecystitis:

cholecystitis is relatively common in pregnancy occurring usually late, 90% of cases have gallstones most of which can be visualized under ultrasound.

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Conventionally cholecystectomy is only necessary in complicated cases such as obstruction, as can cause 15% perinatal mortality.

In advanced pregnancy, cholecystostomy may be possible.

ERCP and endoscopic retrograde sphincterotomy are possible if exposure to radiation is minimized.

In early to middle of second trimester, laparoscopic cholecystostomy carries few problems.

Most patients only require symptomatic relief



6. Pancreatitis

7. Hepatic disorders: e.g.

- pre-eclampsia.**
- acute fatty liver of pregnancy.**
- HELLP syndrome.**

c. Renal system, e.g. cystitis, Pyelonephritis and stones

d. Other uncommon causes include

- sickle cell crisis.**
- malaria.**
- porphyria and**
- diabetic ketoacidosis.**

e. Psychological: diagnosis by exclusion and must be very careful not to miss a physical cause for the abdominal *pain*.



Investigations

- Fetal monitoring
- Urine analysis, MSU: infection, proteinuria in pre-eclampsia
- Full blood count: raised white cell count suggestive of infection, although the white cell count is normally slightly raised in pregnancy
- Liver function tests
- Ultrasound: may demonstrate ectopic pregnancy, abruption, miscarriage
- Laparoscopy to confirm ectopic pregnancy



Management

A thorough assessment of the wellbeing of the mother and fetus, as well as the possible underlying cause is required.

Treatment of cause; urgent hospital referral if uncertain cause, and/or maternal or fetal distress.